

Prevalence and sex ratio. Studies in Europe and in the United States indicate that in the adult population, approximately 18% to 23% of the females and 8% to 11% of the males have at some time had a major depressive episode. It is estimated that 6% of the females and 3% of the males have had a major depressive episode sufficiently severe to require hospitalization.

It is estimated that from 0.4% to 1.2% of the adult population have had Bipolar Disorder. In contrast to Major Depression, Bipolar Disorder is apparently equally common in women and in men.

Familial pattern. Major Affective Disorders are more common among family members than in the general population. This is particularly true for family members of individuals with Bipolar Disorder.

DIAGNOSTIC CRITERIA FOR MAJOR AFFECTIVE DISORDERS

BIPOLAR DISORDER

296.6x Bipolar Disorder, Mixed

Diagnostic criteria for Bipolar Disorder, Mixed

Use fifth-digit coding for manic episode.

- A. Current (or most recent) episode involves the full symptomatic picture of both manic and major depressive episodes (p. 208 and p. 213), intermixed or rapidly alternating every few days.
- B. Depressive symptoms are prominent and last at least a full day.

296.4x Bipolar Disorder, Manic

Diagnostic criteria for Bipolar Disorder, Manic

Currently (or most recently) in a manic episode (p. 208). (If there has been a previous manic episode, the current episode need not meet the full criteria for a manic episode.)

296.5x Bipolar Disorder, Depressed

Diagnostic criteria for Bipolar Disorder, Depressed

- A. Has had one or more manic episodes (p. 208).
- B. Currently (or most recently) in a major depressive episode (p. 213). (If there has been a previous major depressive episode, the current episode of depression need not meet the full criteria for a major depressive episode.)

MAJOR DEPRESSION**296.2x Major Depression, Single Episode****296.3x Major Depression, Recurrent****Diagnostic criteria for Major Depression**

A. One or more major depressive episodes (p. 213).

B. Has never had a manic episode (p. 208).

OTHER SPECIFIC AFFECTIVE DISORDERS

The essential feature is a long-standing illness of at least two years' duration, with either sustained or intermittent disturbance in mood, and associated symptoms. A full affective syndrome is not present, and there are no psychotic features. These disorders usually begin in early adult life, without a clear onset. This category contains two disorders: Cyclothymic Disorder and Dysthymic Disorder. Other terms for these disorders are Cyclothymic and Depressive Personality Disorders.

301.13 Cyclothymic Disorder

The essential feature is a chronic mood disturbance of at least two years' duration, involving numerous periods of depression and hypomania, but not of sufficient severity and duration to meet the criteria for a major depressive or a manic episode (full affective syndrome).

The depressive periods and hypomanic periods may be separated by periods of normal mood lasting as long as several months at a time. In other cases the two types of periods are intermixed or alternate.

During the affective periods there are signs of depression (depressed mood or loss of interest or pleasure in all, or almost all, usual activities and pastimes) and hypomania. In addition, during the affective periods there are paired sets of symptoms (see criterion C below). The following pairs of symptoms are particularly common: feelings of inadequacy (during depressed periods) and inflated self-esteem (during hypomanic periods); social withdrawal and uninhibited people-seeking; sleeping too much and decreased need for sleep; diminished productivity at work and increased productivity, often associated with unusual and self-imposed working hours; decreased attention or concentration and sharpened and unusually creative thinking.

Associated features. Associated features are similar to those of manic episode (p. 207) and major depressive episode (p. 211) except that by definition there are no psychotic features such as delusions, hallucinations, incoherence, or loosening of associations. Substance Abuse is particularly common as a result of

self-treatment with sedatives and alcohol during the depressed periods and the self-indulgent use of stimulants and psychedelics during the hypomanic periods.

Age at onset. Usually early adult life.

Course. The disorder usually begins without clear onset and has a chronic course.

Impairment. Impairment in social and occupational functioning is usually moderate or severe.

Complications. See manic and major depressive episodes (p. 216). Frequently manic and major depressive episodes are complications of this disorder. For this reason some investigators believe that Cyclothymic Disorder is a mild form of Bipolar Disorder.

Predisposing factors. No information.

Prevalence. This disorder was previously assumed to be rare. Recent evidence suggests that among outpatients the disorder may be relatively common, the depressive and hypomanic periods being manifested by loss of interest or pleasure and an expansive or irritable mood rather than by acknowledged depressed and elevated moods.

Sex ratio. The disorder is apparently more common in females.

Familial pattern. Major Depression and Bipolar Disorder are more common among family members of individuals with Cyclothymic Disorder than in the general population.

Differential diagnosis. See manic (p. 208) and major depressive episodes (p. 213). When a **major depressive** or **manic episode** is superimposed on Cyclothymic Disorder, both diagnoses should be made because it is likely that the individual will continue to have Cyclothymic Disorder after recovery from the Major Affective Disorder.

Diagnostic criteria for Cyclothymic Disorder

A. During the past two years, numerous periods during which some symptoms characteristic of both the depressive and the manic syndromes were present, but were not of sufficient severity and duration to meet the criteria for a major depressive or manic episode.

B. The depressive periods and hypomanic periods may be separated by periods of normal mood lasting as long as months at a time, they may be intermixed, or they may alternate.

C. During **depressive** periods there is depressed mood or loss of interest or pleasure in all or almost all, usual activities and pastimes, and at least three of the following:

- (1) insomnia or hypersomnia
- (2) low energy or chronic fatigue
- (3) feelings of inadequacy
- (4) decreased effectiveness or productivity at school, work, or home
- (5) decreased attention, concentration, or ability to think clearly
- (6) social withdrawal
- (7) loss of interest in or enjoyment of sex
- (8) restriction of involvement in pleasurable activities; guilt over past activities
- (9) feeling slowed down
- (10) less talkative than usual
- (11) pessimistic attitude toward the future, or brooding about past events
- (12) tearfulness or crying

During **hypomanic** periods there is an elevated, expansive, or irritable mood and at least three of the following:

- (1) decreased need for sleep
- (2) more energy than usual
- (3) inflated self-esteem
- (4) increased productivity, often associated with unusual and self-imposed working hours
- (5) sharpened and unusually creative thinking
- (6) uninhibited people-seeking (extreme gregariousness)
- (7) hypersexuality without recognition of possibility of painful consequences
- (8) excessive involvement in pleasurable activities with lack of concern for the high potential for painful consequences, e.g., buying sprees, foolish business investments, reckless driving
- (9) physical restlessness
- (10) more talkative than usual
- (11) overoptimism or exaggeration of past achievements
- (12) inappropriate laughing, joking, punning

D. Absence of psychotic features such as delusions, hallucinations, incoherence, or loosening of associations.

E. Not due to any other mental disorder, such as partial remission of Bipolar Disorder. However, Cyclothymic Disorder may precede Bipolar Disorder.

300.40 **Dythymic Disorder (or Depressive Neurosis)**

The essential feature is a chronic disturbance of mood involving either depressed mood or loss of interest or pleasure in all, or almost all, usual activities and

pastimes, and associated symptoms, but not of sufficient severity and duration to meet the criteria for a major depressive episode (full affective syndrome).

For adults, two years' duration is required; for children and adolescents, one year is sufficient.

The depressed mood may be characterized by the individual as feeling sad, blue, down in the dumps, or low. The depressed mood or loss of interest or pleasure may be either relatively persistent or intermittent and separated by periods of normal mood, interest, and pleasure. These normal periods may last a few days to a few weeks. The diagnosis should not be made if an apparently chronic course has been interrupted by a period of normal mood lasting more than a few months.

During the depressive periods there are some of the milder features of the depressive syndrome described as part of a major depressive episode on p. 210 (see criterion D below).

Associated features. Associated features (and age-specific associated features) are similar to those of major depressive episode (p. 211), except that by definition there are no delusions or hallucinations.

Often an associated personality disturbance warrants an additional diagnosis of a Personality Disorder on Axis II.

Age at onset. This disorder usually begins early in adult life, and for this reason was often referred to as Depressive Personality. Although it may begin in childhood or adolescence, in other cases it may begin at a period later in adulthood, in some instances following a Major Depression.

Course. The disorder usually begins without clear onset and has a chronic course.

Impairment and complications. The impairment in social and occupational functioning is usually mild or moderate because of the chronicity rather than the severity of the depressive syndrome. Therefore, hospitalization is rarely required unless there is a suicide attempt or a superimposed Major Affective Disorder. The complications are similar to those of Major Depression, although, because of the chronicity of this disorder, there may be a greater likelihood of developing Substance Abuse.

In children and adolescents social interaction with peers and adults is frequently affected. Children with depression often react negatively or shyly to praise and frequently respond to positive relationships with negative behaviors (sometimes testing, sometimes as manifestations of unexpressed resentment and anger). School performance and progress may be deleteriously affected.

Predisposing factors. Predisposing factors include chronic physical disorder, chronic psychosocial stressors, and another mental disorder, such as a Personality Disorder or an Affective Disorder that does not completely remit and merges imperceptibly into this condition.

In children and adolescents predisposing factors are the presence of Attention Deficit Disorder, Conduct Disorder, Mental Retardation, a severe Specific Developmental Disorder or an inadequate, disorganized, rejecting and chaotic environment.

Prevalence. This disorder is apparently common.

Sex ratio. Among adults the disorder is apparently more common in females. In children it seems to occur equally frequently in both sexes.

Familial pattern. No information.

Differential diagnosis. For a discussion of the differential diagnosis with major depressive episode, see p. 213.

When a Major Depression is in partial remission for a period of two years, Dysthymic Disorder should be considered as an alternative diagnosis to Major Depression in Remission. When a Major Depression is superimposed on Dysthymic Disorder, both diagnoses should be recorded since it is likely that the individual will continue to have the Dysthymic Disorder when he or she has recovered from the Major Depression.

Often the affective features of this disorder are viewed as secondary to an underlying **Personality Disorder**. When an individual meets the criteria for both this disorder and a Personality Disorder, both diagnoses should be made regardless of the causal relationship between the two. This disorder is particularly common in individuals with Borderline, Histrionic and Dependent Personality Disorders.

Normal fluctuations of mood are not as frequent or severe as the depressed mood in Dysthymic Disorder and there is no interference with social functioning.

Chronic mental disorders such as Obsessive Compulsive Disorder or Alcohol Dependence, when associated with depressive symptoms may suggest Dysthymic Disorder. The additional diagnosis of Dysthymic Disorder should be made only if the depressed mood, by virtue of its intensity or effect on functioning, can be clearly distinguished from the individual's usual mood. In children Dysthymic Disorder may be superimposed on **Attention Deficit Disorder**, a **Specific Developmental Disorder**, or an **Organic Mental Disorder**.

Diagnostic criteria for Dysthymic Disorder

A. During the past two years (or one year for children and adolescents) the individual has been bothered most or all of the time by symptoms characteristic of the depressive syndrome but that are not of sufficient severity and duration to meet the criteria for a major depressive episode.

B. The manifestations of the depressive syndrome may be relatively persistent or separated by periods of normal mood lasting a few days to a few weeks, but no more than a few months at a time.

C. During the depressive periods there is either prominent depressed mood (e.g., sad, blue, down in the dumps, low) or marked loss of interest or pleasure in all, or almost all, usual activities and pastimes.

D. During the depressive periods at least three of the following symptoms are present:

- (1) insomnia or hypersomnia
- (2) low energy level or chronic tiredness
- (3) feelings of inadequacy, loss of self-esteem, or self-deprecation
- (4) decreased effectiveness or productivity at school, work, or home
- (5) decreased attention, concentration, or ability to think clearly
- (6) social withdrawal
- (7) loss of interest in or enjoyment of pleasurable activities
- (8) irritability or excessive anger (in children, expressed toward parents or caretakers)
- (9) inability to respond with apparent pleasure to praise or rewards
- (10) less active or talkative than usual, or feels slowed down or restless
- (11) pessimistic attitude toward the future, brooding about past events, or feeling sorry for self
- (12) tearfulness or crying
- (13) recurrent thoughts of death or suicide

E. Absence of psychotic features, such as delusions, hallucinations, or incoherence, or loosening of associations.

F. If the disturbance is superimposed on a preexisting mental disorder, such as Obsessive Compulsive Disorder or Alcohol Dependence, the depressed mood, by virtue of its intensity or effect on functioning, can be clearly distinguished from the individual's usual mood.

ATYPICAL AFFECTIVE DISORDERS

296.70 Atypical Bipolar Disorder

This is a residual category for individuals with manic features that cannot be classified as Bipolar Disorder or as Cyclothymic Disorder. For example, an individual who previously had a major depressive episode and now has an episode of illness with some manic features (hypomanic episode), but not of sufficient severity and duration to meet the criteria for a manic episode. Such cases have been referred to as "Bipolar II."

296.82 Atypical Depression

This is a residual category for individuals with depressive symptoms who cannot be diagnosed as having a Major or Other Specific Affective Disorder or Adjustment Disorder. Examples include the following:

- (1) A distinct and sustained episode of the full depressive syndrome in an individual with Schizophrenia, Residual Type, that develops without an activation of the psychotic symptoms.
- (2) A disorder that fulfills the criteria for Dysthymic Disorder; however, there have been intermittent periods of normal mood lasting more than a few months.
- (3) A brief episode of depression that does not meet the criteria for a Major Affective Disorder and that is apparently not reactive to psychosocial stress, so that it cannot be classified as an Adjustment Disorder.

Anxiety Disorders

In this group of disorders anxiety is either the predominant disturbance, as in Panic Disorder and Generalized Anxiety Disorder, or anxiety is experienced if the individual attempts to master the symptoms, as in confronting the dreaded object or situation in a Phobic Disorder or resisting the obsessions or compulsions in Obsessive Compulsive Disorder. Diagnosis of an Anxiety Disorder is not made if the anxiety is due to another disorder, such as Schizophrenia, an Affective Disorder, or an Organic Mental Disorder.

It has been estimated that from 2% to 4% of the general population has at some time had a disorder that this manual would classify as an Anxiety Disorder.

Panic Disorder, Phobic Disorders and Obsessive Compulsive Disorder are each apparently more common among family members of individuals with each of these disorders than in the general population.

PHOBIC DISORDERS (OR PHOBIC NEUROSES)

The essential feature is persistent and irrational fear of a specific object, activity, or situation that results in a compelling desire to avoid the dreaded object, activity, or situation (the phobic stimulus). The fear is recognized by the individual as excessive or unreasonable in proportion to the actual dangerousness of the object, activity, or situation.

Irrational avoidance of objects, activities, or situations that has an insignificant effect on life adjustment is commonplace. For example, many individuals experience some irrational fear when unable to avoid contact with harmless insects or spiders, but this has no major effect on their lives. However, when the avoidance behavior or fear is a significant source of distress to the individual or interferes with social or role functioning, a diagnosis of a Phobic Disorder is warranted.

The Phobic Disorders are subdivided into three types: Agoraphobia, the most severe and pervasive form; Social Phobia; and Simple Phobia. Both Social and Simple Phobias generally involve a circumscribed stimulus, but Simple Phobia tends to have an earlier onset and better prognosis. When more than one type is present, multiple diagnoses should be made.

Although anxiety related to separation from parental figures is a form of phobic reaction, it is classified as Separation Anxiety Disorder, in the section Disorders Usually First Evident in Infancy, Childhood, or Adolescence (p. 50). Similarly, phobic avoidance limited to sexual activities is classified as a Psychosexual Disorder Not Elsewhere Classified (p. 282).

Although Simple Phobia is the most common type of Phobic Disorder in

the general population, Agoraphobia is the most common among those seeking treatment.

300.21 Agoraphobia with Panic Attacks

300.22 Agoraphobia without Panic Attacks

The essential feature is a marked fear of being alone, or being in public places from which escape might be difficult or help not available in case of sudden incapacitation. Normal activities are increasingly constricted as the fears or avoidance behavior dominate the individual's life. The most common situations avoided involve being in crowds, such as on a busy street or in crowded stores, or being in tunnels, on bridges, on elevators, or on public transportation. Often these individuals insist that a family member or friend accompany them whenever they leave home.

The disturbance is not due to a major depressive episode, Obsessive Compulsive Disorder, Paranoid Personality Disorder, or Schizophrenia.

Often the initial phase of the disorder consists of recurrent panic attacks. (For a description of panic attacks, see p. 230.) The individual develops anticipatory fear of having such an attack and becomes reluctant or refuses to enter a variety of situations that are associated with these attacks. When there is a history of panic attacks (which may or may not be currently present) associated with avoidance behavior, the diagnosis of Agoraphobia with Panic Attacks should be made. Where there is no such history (or this information is lacking), the diagnosis of Agoraphobia without Panic Attacks should be made.

Associated features. Depression, anxiety, rituals, minor "checking" compulsions, or rumination is frequently present.

Age at onset. Most frequently the onset is in the late teens or early 20s, but it can be much later.

Course. The severity of the disturbance waxes and wanes, and periods of complete remission are possible. The activities or situations that the individual dreads may change from day to day.

Impairment. During exacerbations of the illness the individual may be housebound. The avoidance of certain situations, such as being in elevators, may grossly interfere with social and occupational functioning.

Complications. Some individuals attempt to relieve their anxiety with alcohol, barbiturates, or antianxiety medications even to the extent of becoming physiologically dependent on them. Major Depression is another complication.

Predisposing factors. Separation Anxiety Disorder in childhood and sudden object loss apparently predispose to the development of Agoraphobia.

Prevalence. A study of the general population in a small city found that approximately 0.5% of the population had had Agoraphobia at some time.

Sex ratio. The disorder is more frequently diagnosed in women.

Differential diagnosis. In **Schizophrenia**, **Major Depression**, **Obsessive Compulsive Disorder** and **Paranoid Personality Disorder** there may be phobic avoidance of certain situations. The diagnosis of Agoraphobia is not made if a phobia is due to any of these disorders.

Diagnostic criteria for Agoraphobia

A. The individual has marked fear of and thus avoids being alone or in public places from which escape might be difficult or help not available in case of sudden incapacitation, e.g., crowds, tunnels, bridges, public transportation.

B. There is increasing constriction of normal activities until the fears or avoidance behavior dominate the individual's life.

C. Not due to a major depressive episode, Obsessive Compulsive Disorder, Paranoid Personality Disorder, or Schizophrenia.

300.23 Social Phobia

The essential feature is a persistent, irrational fear of, and compelling desire to avoid, situations in which the individual may be exposed to scrutiny by others. There is also fear that the individual may behave in a manner that will be humiliating or embarrassing. Marked anticipatory anxiety occurs if the individual is confronted with the necessity of entering into such a situation, and he or she therefore attempts to avoid it. The disturbance is a significant source of distress and is recognized by the individual as excessive or unreasonable. It is not due to any other mental disorder. Examples of Social Phobias are fears of speaking or performing in public, using public lavatories, eating in public, and writing in the presence of others. Generally an individual has only one Social Phobia.

Usually the individual is aware that the fear is that others will detect signs of anxiety in the phobic situation. For example, the individual with a fear of writing in the presence of others is concerned that others may detect a hand tremor. A vicious cycle may be created in which the irrational fear generates anxiety that impairs performance, thus providing an apparent justification for avoiding the phobic situation.

Associated features. Considerable unfocused or generalized anxiety may also be present. Agoraphobia or Simple Phobia may coexist with Social Phobia.

Age at onset. The disorder often begins in late childhood or early adolescence.

Course. The disorder is usually chronic, and may undergo exacerbation

when the anxiety impairs performance of the feared activity. This then leads to increased anxiety, which strengthens the phobic avoidance.

Impairment. Unless the disorder is severe, it is rarely, in itself, incapacitating. However, considerable inconvenience may result from the need to avoid the phobic situation, e.g., avoiding a trip if it would necessitate the use of a public lavatory. Fear of public speaking may interfere with professional advancement.

Complications. Individuals with this disorder are prone to the episodic abuse of alcohol, barbiturates, and antianxiety medications, which they may use to relieve their anxiety.

Prevalence. The disorder is apparently relatively rare.

Predisposing factors, sex ratio, and familial pattern. No information.

Differential diagnosis. Avoidance of certain social situations that are normally a source of some distress, which is common in many individuals with "normal" fear of public speaking, does not justify a diagnosis of Social Phobia. In **Schizophrenia**, **Major Depression**, **Obsessive Compulsive Disorder**, and **Paranoid** and **Avoidant Personality Disorders**, there may be marked anxiety and avoidance of certain social situations. However, the diagnosis of Social Phobia is not made if the phobia is due to any of these disorders.

In **Simple Phobia** there is also a circumscribed phobic stimulus, but it is not a social situation involving the possibility of humiliation or embarrassment.

Diagnostic criteria for Social Phobia

A. A persistent, irrational fear of, and compelling desire to avoid, a situation in which the individual is exposed to possible scrutiny by others and fears that he or she may act in a way that will be humiliating or embarrassing.

B. Significant distress because of the disturbance and recognition by the individual that his or her fear is excessive or unreasonable.

C. Not due to another mental disorder, such as Major Depression or Avoidant Personality Disorder.

300.29 Simple Phobia

The essential feature is a persistent, irrational fear of, and compelling desire to avoid, an object or a situation other than being alone or in public places away from home (Agoraphobia), or of humiliation or embarrassment in certain social situations (Social Phobia). Thus, this is a residual category of Phobic Disorder. This disturbance is a significant source of distress, and the individual recognizes

that his or her fear is excessive or unreasonable. The disturbance is not due to another mental disorder.

Simple Phobias are sometimes referred to as "specific" phobias. The most common Simple Phobias in the general population, though not necessarily among those seeking treatment, involve animals, particularly dogs, snakes, insects, and mice. Other Simple Phobias are claustrophobia (fear of closed spaces) and acrophobia (fear of heights).

Associated features. When suddenly exposed to the phobic stimulus, the individual becomes overwhelmingly fearful and may experience symptoms identical with those of a panic attack (p. 230). Because of anticipatory anxiety, the individual will often try to gain considerable information before entering situations in which the phobic stimulus may be encountered.

Age at onset. Age at onset varies, but animal phobias nearly always begin in childhood.

Course. Most simple phobias that start in childhood disappear without treatment. However, those that persist into adulthood rarely remit without treatment.

Impairment. Impairment may be minimal if the phobic object is rare and easily avoided, such as fear of snakes in someone living in the city. Impairment may be considerable if the phobic object is common and cannot be avoided, such as a fear of elevators in someone living in a large city who must use elevators at work.

Complications and predisposing factors. No information.

Prevalence. Simple Phobias may be common; but since they rarely result in marked impairment, individuals with Simple Phobia rarely seek treatment.

Sex ratio. The disorder is more often diagnosed in women.

Differential diagnosis. In **Schizophrenia** certain activities may be avoided in response to delusions. Similarly, in **Obsessive Compulsive Disorder** phobic avoidance of certain situations that are associated with anxiety about dirt or contamination is frequent. The diagnosis of Simple Phobia should not be made in either case.

Diagnostic criteria for Simple Phobia

A. A persistent, irrational fear of, and compelling desire to avoid, an object or a situation other than being alone, or in public places away from home (Agoraphobia), or of humiliation or embarrassment in certain social situations (Social Phobia). Phobic objects are often animals, and phobic situations frequently involve heights or closed spaces.

B. Significant distress from the disturbance and recognition by the individual that his or her fear is excessive or unreasonable.

C. Not due to another mental disorder, such as Schizophrenia or Obsessive Compulsive Disorder.

ANXIETY STATES (OR ANXIETY NEUROSES)

300.01 Panic Disorder

The essential features are recurrent panic (anxiety) attacks that occur at times unpredictably, though certain situations, e.g., driving a car, may become associated with a panic attack. The same clinical picture occurring during marked physical exertion or a life-threatening situation is not termed a panic attack.

The panic attacks are manifested by the sudden onset of intense apprehension, fear, or terror, often associated with feelings of impending doom. The most common symptoms experienced during an attack are dyspnea; palpitations; chest pain or discomfort; choking or smothering sensations; dizziness, vertigo, or unsteady feelings; feelings of unreality (depersonalization or derealization); paresthesias; hot and cold flashes; sweating; faintness; trembling or shaking; and fear of dying, going crazy, or doing something uncontrolled during the attack. Attacks usually last minutes; more rarely, hours.

A common complication of this disorder is the development of an anticipatory fear of helplessness or loss of control during a panic attack, so that the individual becomes reluctant to be alone or in public places away from home. When many situations of the kind are avoided the diagnosis of Agoraphobia with Panic Attacks should be made (p. 226) rather than Panic Disorder.

Associated features. The individual often develops varying degrees of nervousness and apprehension between attacks. This nervousness and apprehension is characterized by the usual manifestations of apprehensive expectation, vigilance and scanning, motor tension, and autonomic hyperactivity.

Age at onset. The disorder often begins in late adolescence or early adult life, but may occur initially in mid-adult life.

Course. The disorder may be limited to a single brief period lasting several weeks or months, recur several times, or become chronic.

Impairment. Except when the disorder is severe or complicated by Agoraphobia, it is rarely incapacitating.

Complications. The complication of Agoraphobia with Panic Attacks has been mentioned above. Other complications include abuse of alcohol and anti-anxiety medications, and Depressive Disorders.

Predisposing factors. Separation Anxiety Disorder in childhood and sudden object loss apparently predispose to the development of this disorder.

Prevalence. The disorder is apparently common.

Sex ratio. This condition is diagnosed much more commonly in women.

Differential diagnosis. Physical disorders such as hypoglycemia, pheochromocytoma, and hyperthyroidism, all of which can cause similar symptoms, must be ruled out.

In **Withdrawal** from some substances, such as **barbiturates**, and in some **Substance Intoxications**, such as due to **caffeine** or **amphetamines**, there may be panic attacks. Panic Disorder should not be diagnosed when the panic attacks are due to Substance-induced Organic Mental Disorder.

In **Schizophrenia**, **Major Depression**, or **Somatization Disorder** panic attacks may occur. However, the diagnosis of Panic Disorder is not made if the panic attacks are due to these other disorders.

Generalized Anxiety Disorder may be confused with the chronic anxiety that often develops between panic attacks in Panic Disorder. A history of recurrent panic attacks precludes Generalized Anxiety Disorder.

In **Simple** or **Social Phobia**, the individual may develop panic attacks if exposed to the phobic stimulus. However, in Panic Disorder, the individual is never certain which situations provoke panic attacks.

Diagnostic criteria for Panic Disorder

A. At least three panic attacks within a three-week period in circumstances other than during marked physical exertion or in a life-threatening situation. The attacks are not precipitated only by exposure to a circumscribed phobic stimulus.

B. Panic attacks are manifested by discrete periods of apprehension or fear, and at least four of the following symptoms appear during each attack:

- (1) dyspnea
- (2) palpitations
- (3) chest pain or discomfort
- (4) choking or smothering sensations
- (5) dizziness, vertigo, or unsteady feelings
- (6) feelings of unreality
- (7) paresthesias (tingling in hands or feet)
- (8) hot and cold flashes
- (9) sweating
- (10) faintness
- (11) trembling or shaking

(12) fear of dying, going crazy, or doing something uncontrolled during an attack

C. Not due to a physical disorder or another mental disorder, such as Major Depression, Somatization Disorder, or Schizophrenia.

D. The disorder is not associated with Agoraphobia (p. 227).

300.02 Generalized Anxiety Disorder

The essential feature is generalized, persistent anxiety of at least one month's duration without the specific symptoms that characterize Phobic Disorders (phobias), Panic Disorder (panic attacks), or Obsessive Compulsive Disorder (obsessions or compulsions). The diagnosis is not made if the disturbance is due to another physical or mental disorder, such as hyperthyroidism or Major Depression.

Although the specific manifestations of the anxiety vary from individual to individual, generally there are signs of motor tension, autonomic hyperactivity, apprehensive expectation, and vigilance and scanning.

(1) *Motor tension.* Shakiness, jitteriness, jumpiness, trembling, tension, muscle aches, fatigability, and inability to relax are common complaints. There may also be eyelid twitch, furrowed brow, strained face, fidgeting, restlessness, easy startle, and sighing respiration.

(2) *Autonomic hyperactivity.* There may be sweating, heart pounding or racing, cold, clammy hands, dry mouth, dizziness, light-headedness, paresthesias (tingling in hands or feet), upset stomach, hot or cold spells, frequent urination, diarrhea, discomfort in the pit of the stomach, lump in the throat, flushing, pallor, and high resting pulse and respiration rate.

(3) *Apprehensive expectation.* The individual is generally apprehensive and continually feels anxious, worries, ruminates, and anticipates that something bad will happen to himself or herself (e.g., fear of fainting, losing control, dying) or to others (e.g., family members may become ill or injured in an accident).

(4) *Vigilance and scanning.* Apprehensive expectation may cause hyperattentiveness so that the individual feels "on edge," impatient, or irritable. There may be complaints of distractibility, difficulty in concentrating, insomnia, difficulty in falling asleep, interrupted sleep, and fatigue on awakening.

Associated features. Mild depressive symptoms are common.

Impairment. Impairment in social or occupational functioning is rarely more than mild.

Complications. Abuse of alcohol, barbiturates, and antianxiety medications is common.

Age at onset, course, predisposing factors, prevalence, sex ratio, and familial pattern. No information.

Differential diagnosis. Physical disorders, such as hyperthyroidism, and Organic Mental Disorders, such as Caffeine Intoxication, must be ruled out.

In **Adjustment Disorder with Anxious Mood**, the full symptom picture required to meet the criteria for Generalized Anxiety Disorder is generally not present, the duration of the disturbance is usually less than a month, and a psychosocial stressor must be recognized.

In **Schizophrenia, Depressive Disorders, Hypochondriasis, Obsessive Compulsive Disorder**, and many other mental disorders, generalized and persistent anxiety is often a prominent symptom. The diagnosis of Generalized Anxiety Disorder is not made if the anxiety is judged to be due to another mental disorder.

In **Panic Disorder** there is often severe chronic anxiety between panic attacks. If the panic attacks are overlooked, an incorrect diagnosis of Generalized Anxiety Disorder may be made.

Diagnostic criteria for Generalized Anxiety Disorder

A. Generalized, persistent anxiety is manifested by symptoms from three of the following four categories:

- (1) *motor tension*: shakiness, jitteriness, jumpiness, trembling, tension, muscle aches, fatigability, inability to relax, eyelid twitch, furrowed brow, strained face, fidgeting, restlessness, easy startle
- (2) *autonomic hyperactivity*: sweating, heart pounding or racing, cold, clammy hands, dry mouth, dizziness, light-headedness, paresthesias (tingling in hands or feet), upset stomach, hot or cold spells, frequent urination, diarrhea, discomfort in the pit of the stomach, lump in the throat, flushing, pallor, high resting pulse and respiration rate
- (3) *apprehensive expectation*: anxiety, worry, fear, rumination, and anticipation of misfortune to self or others
- (4) *vigilance and scanning*: hyperattentiveness resulting in distractibility, difficulty in concentrating, insomnia, feeling "on edge," irritability, impatience

B. The anxious mood has been continuous for at least one month.

C. Not due to another mental disorder, such as a Depressive Disorder or Schizophrenia.

D. At least 18 years of age.

300.30 Obsessive Compulsive Disorder (or Obsessive Compulsive Neurosis)

The essential features are recurrent obsessions or compulsions. *Obsessions* are recurrent, persistent ideas, thoughts, images, or impulses that are ego-dystonic, that is, they are not experienced as voluntarily produced, but rather as thoughts that invade consciousness and are experienced as senseless or repugnant. Attempts are made to ignore or suppress them. *Compulsions* are repetitive and seemingly purposeful behaviors that are performed according to certain rules or in a stereotyped fashion. The behavior is not an end in itself, but is designed to produce or to prevent some future event or situation. However, the activity is not connected in a realistic way with what it is designed to produce or prevent, or may be clearly excessive. The act is performed with a sense of subjective compulsion coupled with a desire to resist the compulsion (at least initially). The individual generally recognizes the senselessness of the behavior (this may not be true for young children) and does not derive pleasure from carrying out the activity, although it provides a release of tension.

The most common obsessions are repetitive thoughts of violence (e.g., killing one's child), contamination (e.g., becoming infected by shaking hands), and doubt (e.g., repeatedly wondering whether one has performed some action, such as having hurt someone in a traffic accident). The most common compulsions involve hand-washing, counting, checking, and touching.

When the individual attempts to resist a compulsion, there is a sense of mounting tension that can be immediately relieved by yielding to the compulsion. In the course of the illness, after repeated failure at resisting the compulsions, the individual may give in to them and no longer experience a desire to resist them.

Associated features. Depression and anxiety are common. Frequently there is phobic avoidance of situations that involve the content of the obsessions, such as dirt or contamination.

Age at onset. Although the disorder usually begins in adolescence or early adulthood, it may begin in childhood.

Course. The course is usually chronic, with waxing and waning of symptoms.

Impairment. Impairment is generally moderate to severe. In some cases compulsions may become the major life activity.

Complications. Complications include Major Depression and the abuse of alcohol and antianxiety medications.

Predisposing factors. No information.

Prevalence. The disorder is apparently rare in the general population.

Sex ratio. This disorder is equally common in males and in females.

Differential diagnosis. Some activities, such as **eating, sexual behavior (e.g., Paraphilias), gambling, or drinking, when engaged in excessively** may be referred to as "compulsive". However, these activities are not true compulsions, because the individual derives pleasure from the particular activity and may wish to resist it only because of its secondary deleterious consequences.

Obsessive brooding, rumination or preoccupation, i.e., excessive and repetitive thinking about real or potentially unpleasant circumstances, or indecisive consideration of alternatives lacks the quality of being ego-dystonic, because the individual generally regards the ideation as meaningful, although possibly excessive. Therefore, these are not true obsessions.

In **Schizophrenia**, stereotyped behavior is common, but can be explained by delusions rather than as being ego-dystonic. Obsessions and compulsions sometimes occur transiently during the prodromal phase of Schizophrenia. In such cases the diagnosis of Obsessive Compulsive Disorder is not made. **Tourette's Disorder, Schizophrenia, Major Depression** and, very rarely, **Organic Mental Disorder** may have obsessions and compulsions as symptoms, but in such instances the diagnosis Obsessive Compulsive Disorder is not made. However, Obsessive Compulsive Disorder may precede the development of a Major Depression, in which case both diagnoses should be recorded.

Diagnostic criteria for Obsessive Compulsive Disorder

A. Either obsessions or compulsions:

Obsessions: recurrent, persistent ideas, thoughts, images, or impulses that are ego-dystonic, i.e., they are not experienced as voluntarily produced, but rather as thoughts that invade consciousness and are experienced as senseless or repugnant. Attempts are made to ignore or suppress them.

Compulsions: repetitive and seemingly purposeful behaviors that are performed according to certain rules or in a stereotyped fashion. The behavior is not an end in itself, but is designed to produce or prevent some future event or situation. However, either the activity is not connected in a realistic way with what it is designed to produce or prevent, or may be clearly excessive. The act is performed with a sense of subjective compulsion coupled with a desire to resist the compulsion (at least initially). The individual generally recognizes the senselessness of the behavior (this may not be true for young children) and does not derive pleasure from carrying out the activity, although it provides a release of tension.

B. The obsessions or compulsions are a significant source of distress to the individual or interfere with social or role functioning.

C. Not due to another mental disorder, such as Tourette's Disorder, Schizophrenia, Major Depression, or Organic Mental Disorder.

308.30 Post-traumatic Stress Disorder, Acute**309.81 Post-traumatic Stress Disorder, Chronic or Delayed**

The essential feature is the development of characteristic symptoms following a psychologically traumatic event that is generally outside the range of usual human experience.

The characteristic symptoms involve reexperiencing the traumatic event; numbing of responsiveness to, or reduced involvement with, the external world; and a variety of autonomic, dysphoric, or cognitive symptoms.

The stressor producing this syndrome would evoke significant symptoms of distress in most people, and is generally outside the range of such common experiences as simple bereavement, chronic illness, business losses, or marital conflict. The trauma may be experienced alone (rape or assault) or in the company of groups of people (military combat). Stressors producing this disorder include natural disasters (floods, earthquakes), accidental man-made disasters (car accidents with serious physical injury, airplane crashes, large fires), or deliberate man-made disasters (bombing, torture, death camps). Some stressors frequently produce the disorder (e.g., torture) and others produce it only occasionally (e.g., car accidents). Frequently there is a concomitant physical component to the trauma which may even involve direct damage to the central nervous system (e.g., malnutrition, head trauma). The disorder is apparently more severe and longer lasting when the stressor is of human design. The severity of the stressor should be recorded and the specific stressor may be noted on Axis IV (p. 26).

The traumatic event can be reexperienced in a variety of ways. Commonly the individual has recurrent painful, intrusive recollections of the event or recurrent dreams or nightmares during which the event is reexperienced. In rare instances there are dissociativelike states, lasting from a few minutes to several hours or even days, during which components of the event are relived and the individual behaves as though experiencing the event at that moment. Such states have been reported in combat veterans. Diminished responsiveness to the external world, referred to as "psychic numbing" or "emotional anesthesia," usually begins soon after the traumatic event. A person may complain of feeling detached or estranged from other people, that he or she has lost the ability to become interested in previously enjoyed significant activities, or that the ability to feel emotions of any type, especially those associated with intimacy, tenderness, and sexuality, is markedly decreased.

After experiencing the stressor, many develop symptoms of excessive autonomic arousal, such as hyperalertness, exaggerated startle response, and difficulty falling asleep. Recurrent nightmares during which the traumatic event is relived and which are sometimes accompanied by middle or terminal sleep disturbance may be present. Some complain of impaired memory or difficulty in concentrating or completing tasks. In the case of a life-threatening trauma shared with others, survivors often describe painful guilt feelings about surviving when many did not, or about the things they had to do in order to survive. Activities or situations that may arouse recollections of the traumatic event are

often avoided. Symptoms characteristic of Post-traumatic Stress Disorder are often intensified when the individual is exposed to situations or activities that resemble or symbolize the original trauma (e.g., cold snowy weather or uniformed guards for death-camp survivors, hot, humid weather for veterans of the South Pacific).

Associated features. Symptoms of depression and anxiety are common, and in some instances may be sufficiently severe to be diagnosed as an Anxiety or Depressive Disorder. Increased irritability may be associated with sporadic and unpredictable explosions of aggressive behavior, upon even minimal or no provocation. The latter symptom has been reported to be particularly characteristic of war veterans with this disorder. Impulsive behavior can occur, such as sudden trips, unexplained absences, or changes in life-style or residence. Survivors of death camps sometimes have symptoms of an Organic Mental Disorder, such as failing memory, difficulty in concentrating, emotional lability, autonomic lability, headache, and vertigo.

Age at onset. The disorder can occur at any age, including during childhood.

Course and subtypes. Symptoms may begin immediately or soon after the trauma. It is not unusual, however, for the symptoms to emerge after a latency period of months or years following the trauma.

When the symptoms begin within six months of the trauma and have not lasted more than six months, the acute subtype is diagnosed, and the prognosis for remission is good. If the symptoms either develop more than six months after the trauma or last six months or more, the chronic or delayed subtype is diagnosed.

Impairment and complications. Impairment may either be mild or affect nearly every aspect of life. Phobic avoidance of situations or activities resembling or symbolizing the original trauma may result in occupational or recreational impairment. "Psychic numbing" may interfere with interpersonal relationships, such as marriage or family life. Emotional lability, depression, and guilt may result in self-defeating behavior or suicidal actions. Substance Use Disorders may develop.

Predisposing factors. Preexisting psychopathology apparently predisposes to the development of the disorder.

Prevalence. No information.

Sex ratio and familial pattern. No information.

Differential diagnosis. If an Anxiety, Depressive, or Organic Mental Disorder develops following the trauma, these diagnoses should also be made.

In **Adjustment Disorder**, the stressor is usually less severe and within the range of common experience; and the characteristic symptoms of Post-traumatic Stress Disorder, such as reexperiencing the trauma, are absent.

Diagnostic criteria for Post-traumatic Stress Disorder

A. Existence of a recognizable stressor that would evoke significant symptoms of distress in almost everyone.

B. Reexperiencing of the trauma as evidenced by at least one of the following:

- (1) recurrent and intrusive recollections of the event
- (2) recurrent dreams of the event
- (3) sudden acting or feeling as if the traumatic event were reoccurring, because of an association with an environmental or ideational stimulus

C. Numbing of responsiveness to or reduced involvement with the external world, beginning some time after the trauma, as shown by at least one of the following:

- (1) markedly diminished interest in one or more significant activities
- (2) feeling of detachment or estrangement from others
- (3) constricted affect

D. At least two of the following symptoms that were not present before the trauma:

- (1) hyperalertness or exaggerated startle response
- (2) sleep disturbance
- (3) guilt about surviving when others have not, or about behavior required for survival
- (4) memory impairment or trouble concentrating
- (5) avoidance of activities that arouse recollection of the traumatic event
- (6) intensification of symptoms by exposure to events that symbolize or resemble the traumatic event

SUBTYPES

Post-traumatic Stress Disorder, Acute

A. Onset of symptoms within six months of the trauma.

B. Duration of symptoms less than six months.

Post-traumatic Stress Disorder, Chronic or Delayed

Either of the following, or both:

- (1) duration of symptoms six months or more (chronic)
- (2) onset of symptoms at least six months after the trauma (delayed)

300.00 Atypical Anxiety Disorder

This category should be used when the individual appears to have an Anxiety Disorder that does not meet the criteria for any of the above specified conditions.

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Somatoform Disorders

The essential features of this group of disorders are physical symptoms suggesting physical disorder (hence, Somatoform) for which there are no demonstrable organic findings or known physiological mechanisms and for which there is positive evidence, or a strong presumption, that the symptoms are linked to psychological factors or conflicts. Unlike Factitious Disorder or Malingering, the symptom production in Somatoform Disorders is not under voluntary control, i.e., the individual does not experience the sense of controlling the production of the symptoms. Although the symptoms of Somatoform Disorders are "physical," the specific pathophysiological processes involved are not demonstrable or understandable by existing laboratory procedures and are conceptualized most clearly using psychological constructs. For that reason, these disorders are not classified as "physical disorders."

The first disorder in this category is Somatization Disorder, a common and chronic polysymptomatic disorder that begins early in life and that was previously referred to as either Hysteria or Briquet's Syndrome. The second disorder is Conversion Disorder, which, as defined here, is relatively uncommon. This diagnosis is to be used only when conversion symptoms are the predominant disturbance and are not symptomatic of another disorder. Psychogenic Pain Disorder is characterized by psychologically induced pain not attributable to any other mental or physical disorder. Hypochondriasis involves preoccupation with the fear or belief of having a serious disease. Finally, Atypical Somatoform Disorder is the term applied to physical symptoms without an organic basis that do not fit the criteria for any specific Somatoform Disorder.

300.81 Somatization Disorder

The essential features are recurrent and multiple somatic complaints of several years' duration for which medical attention has been sought but which are apparently not due to any physical disorder. The disorder begins before the age of 30 and has a chronic but fluctuating course.

Complaints are often presented in a dramatic, vague, or exaggerated way, or are part of a complicated medical history in which many physical diagnoses have been considered. The individuals frequently receive medical care from a number of physicians, sometimes simultaneously. (Although most people without mental disorders at various times have aches and pains and other physical complaints, they rarely bring them to medical attention.) Complaints invariably involve the following organ systems or types of symptoms: conversion or pseudoneurological (e.g., paralysis, blindness), gastrointestinal (e.g., abdominal pain), female reproductive (e.g., painful menstruation), psychosexual (e.g., sexual indifference), pain (e.g., back pain), and cardiopulmonary (e.g., dizziness).

Associated features. Anxiety and depressed mood are extremely common. In fact, many individuals with this disorder who seek mental health care do so because of the depressive symptoms, which include suicide threats and attempts. Antisocial behavior and occupational, interpersonal, and marital difficulties are common. Hallucinations are also reported; this is usually the hallucination of hearing one's name called without impairment of reality testing. Histrionic Personality Disorder and, more rarely, Antisocial Personality Disorder often are also present.

Age at onset. Symptoms usually begin in the teen years or, rarely, in the 20s. Menstrual difficulties may be one of the earliest symptoms in females, although preadolescents and adolescents may present with seizures, depressive symptoms, headache, abdominal pain, or a plethora of other physical symptoms.

Course. This is a chronic but fluctuating disorder that rarely remits spontaneously. A year seldom passes without some medical attention.

Impairment and complications. Because of constant seeking out of doctors, numerous medical evaluations are undergone, both in and out of the hospital; and there is frequently unwitting submission to unnecessary surgery. These individuals run the risk of Substance Use Disorders involving various prescribed medicines. Because of depressive symptoms, they may experience long periods of incapacity and frequent suicidal threats and attempts. Completed suicide, when it occurs, is usually associated with Substance Abuse. People with this disorder often lead lives as chaotic and complicated as their medical histories.

Predisposing factors. No information.

Prevalence and sex ratio. Approximately 1% of females have this disorder. The disorder is rarely diagnosed in males.

Familial pattern. This disorder and Antisocial Personality Disorder are more common among family members than in the general population.

Differential diagnosis. It is necessary to rule out physical disorders that present with vague, multiple, and confusing somatic symptoms, e.g., hyperparathyroidism, porphyria, multiple sclerosis, and systemic lupus erythematosus. The onset of multiple physical symptoms late in life is almost always due to physical disease.

Schizophrenia with multiple somatic delusions needs to be differentiated from the nondelusional somatic complaints of individuals with Somatization Disorder. Rarely, individuals with Somatization Disorder also have Schizophrenia, in which case both diagnoses should be noted.

Dysthymic Disorder and **Generalized Anxiety Disorder** are not diagnosed in individuals who have Somatization Disorder since mild depressive and anxiety symptoms are so ubiquitous in Somatization Disorder. On the other hand, a superimposed **Major Depression** should be diagnosed if there is a full and per-

sistent affective syndrome that can be clearly distinguished from the individual's usual condition.

In **Panic Disorder** there are also cardiopulmonary symptoms, but these occur only in the context of panic attacks. However, Panic Disorder may coexist with Somatization Disorder, in which case both diagnoses should be made.

In **Conversion Disorder** one or more conversion symptoms occur in the absence of the full clinical picture of Somatization Disorder.

In **Factitious Disorder with Physical Symptoms** the individual has voluntary control of the symptoms.

Diagnostic criteria for Somatization Disorder

A. A history of physical symptoms of several years' duration beginning before the age of 30.

B. Complaints of at least 14 symptoms for women and 12 for men, from the 37 symptoms listed below. To count a symptom as present the individual must report that the symptom caused him or her to take medicine (other than aspirin), alter his or her life pattern, or see a physician. The symptoms, in the judgment of the clinician, are not adequately explained by physical disorder or physical injury, and are not side effects of medication, drugs or alcohol. The clinician need not be convinced that the symptom was actually present, e.g., that the individual actually vomited throughout her entire pregnancy; report of the symptom by the individual is sufficient.

Sickly: Believes that he or she has been sickly for a good part of his or her life.

Conversion or pseudoneurological symptoms: Difficulty swallowing, loss of voice, deafness, double vision, blurred vision, blindness, fainting or loss of consciousness, memory loss, seizures or convulsions, trouble walking, paralysis or muscle weakness, urinary retention or difficulty urinating.

Gastrointestinal symptoms: Abdominal pain, nausea, vomiting spells (other than during pregnancy), bloating (gassy), intolerance (e.g., gets sick) of a variety of foods, diarrhea.

Female reproductive symptoms: Judged by the individual as occurring more frequently or severely than in most women: painful menstruation, menstrual irregularity, excessive bleeding, severe vomiting throughout pregnancy or causing hospitalization during pregnancy.

Psychosexual symptoms: For the major part of the individual's life after opportunities for sexual activity: sexual indifference, lack of pleasure during intercourse, pain during intercourse.

Pain: Pain in back, joints, extremities, genital area (other than during intercourse); pain on urination; other pain (other than headaches).

Cardiopulmonary symptoms: Shortness of breath, palpitations, chest pain, dizziness.

300.11 Conversion Disorder (or Hysterical Neurosis, Conversion Type)

The essential feature is a clinical picture in which the predominant disturbance is a loss of or alteration in physical functioning that suggests physical disorder but which instead is apparently an expression of a psychological conflict or need. The disturbance is not under voluntary control, and after appropriate investigation cannot be explained by any physical disorder or known pathophysiological mechanism. Conversion Disorder is not diagnosed when conversion symptoms are limited to pain (see Psychogenic Pain Disorder, p. 247) or to a disturbance in sexual functioning (see Psychosexual Dysfunctions, p. 275) or are part of Somatization Disorder (p. 241).

The most obvious and "classic" conversion symptoms are those that suggest neurological disease, such as paralysis, aphonia, seizures, coordination disturbance, akinesia, dyskinesia, blindness, tunnel vision, anosmia, anesthesia, and paresthesia. More rarely, conversion symptoms may involve the autonomic or endocrine system. Vomiting as a conversion symptom can represent revulsion and disgust. Pseudocyesis (false pregnancy) can represent both a wish for, and a fear of, pregnancy.

The definition of this disorder is unique in this classification in that it implies specific mechanisms to account for the disturbance. Two mechanisms have been suggested to explain what the individual derives from having a conversion symptom.

In one mechanism, the individual achieves "primary gain" by keeping an internal conflict or need out of awareness. In such cases there is a temporal relationship between an environmental stimulus that is apparently related to a psychological conflict or need and the initiation or exacerbation of the symptom. For example, after an argument, inner conflict about the expression of rage may be expressed as "aphonia" or as a "paralysis" of the arm; or if the individual views a traumatic event, a conflict about acknowledging that event may be expressed as "blindness." In such cases the symptom has a symbolic value that is a representation and partial solution of the underlying psychological conflict.

In the other mechanism the individual achieves "secondary gain" by avoiding a particular activity that is noxious to him or her or by getting support from the environment that otherwise might not be forthcoming. For example, with a "paralyzed" hand a soldier can avoid firing a gun; or a person with marked dependency needs may develop "blindness" or inability to walk or stand, even though all leg movements can be performed normally (astasia-abasia), to prevent desertion by a spouse.

A conversion symptom is likely to involve a single symptom during a given episode, but may vary in site and nature if there are subsequent episodes.

Associated features. Usually the symptom develops in a setting of extreme psychological stress and appears suddenly. Histrionic personality traits (see p. 313) are common, but not invariably present. "La belle indifference," an attitude toward the symptom that suggests a relative lack of concern, out of keeping with the severe nature of the impairment, is sometimes present. This feature has little diagnostic value, however, since it is also found in some seriously ill medical patients who are stoic about their situation.

Age at onset. The usual age at onset is adolescence or early adulthood, but the symptom may appear for the first time during middle age or even in the later decades of life.

Course. The course of Conversion Disorder (as distinct from conversion symptoms that are part of other disorders, such as Somatization Disorder) is unknown, but probably is usually of short duration, with abrupt onset and resolution. Some individuals given an initial diagnosis of conversion symptoms are later found to have a neurological disorder. Apparently, in some of these instances the earliest symptoms of the neurological disorder predisposed to the development of a concomitant conversion symptom. In other instances the original diagnosis of a conversion symptom was incorrect and represented a missed diagnosis of true organic pathology.

Impairment and complications. The effect of the disorder on the individual's life is usually marked and frequently prevents normal life activities. Prolonged loss of function may produce real and serious complications, such as contractures or disuse atrophy from conversion paralysis. When associated with Dependent Personality Disorder, the conversion symptom may enhance the development of a chronic sick role. Unnecessary diagnostic or therapeutic medical procedures may themselves produce disfigurement or incapacity.

Predisposing factors. Antecedent physical disorder (which may provide a prototype for the symptoms, e.g., pseudoseizures in individuals with epilepsy), exposure to other individuals with real physical symptoms or conversion symptoms, and extreme psychosocial stress (e.g., warfare or the recent death of a significant figure) are predisposing factors. Histrionic and Dependent Personality Disorders also predispose to the development of the disorder.

Prevalence. Although Conversion Disorder was apparently common several decades ago, it is now rarely encountered. Most cases are seen on neurology or orthopedic wards and in military settings, especially in time of warfare.

Sex ratio. No definite information is available; but one particular conversion symptom, globus hystericus, the feeling of a lump in the throat that interferes with swallowing, is apparently more common in women.

Familial pattern. No information.

Differential diagnosis. Some physical disorders that present with vague, multiple, somatic symptoms, such as multiple sclerosis or systemic lupus erythematosus, may early in their course be misdiagnosed as conversion symptoms. A diagnosis of Conversion Disorder is suggested if the symptoms are inconsistent with the actual known physical disorder—for example, motor signs of good function in a supposedly paralyzed part, or complaints obviously inconsistent with the anatomic distribution of the nervous system. Another example would be “anesthesia” of the hand conforming to the concept of the hand rather than to the functional area served by a specific part of the nervous system. In another example, an individual with conversion blindness may be found to have normal pupillary responses and evoked potentials as measured by an EEG. Resolution of symptoms through suggestion, hypnosis, or narcoanalysis suggests a conversion symptom. Temporary improvement due to suggestion has little diagnostic value since this may also occur with true physical illness.

In **undiagnosed physical disorder** physical symptoms are present that are not explained by a known physical disorder, but there is no evidence that the symptom serves a psychological purpose. **Physical disorders in which psychological factors often play an important role, such as irritable colon or bronchial asthma**, should not be diagnosed as Conversion Disorders, since demonstrable organic pathology or a pathophysiological mechanism that accounts for the disorder is present.

Somatization Disorder and, more rarely, **Schizophrenia** may have conversion symptoms. However, the diagnosis of Conversion Disorder should not be made when such symptoms are due to either of these more pervasive disorders.

For many of the **Psychosexual Dysfunctions**, it is difficult to determine whether the symptom, such as impotence in the male or lack of sexual excitement in the female, represents a physiological reaction to anxiety or a direct expression of a psychological conflict or need (conversion symptom). For this reason, and in order to group all of the sexual disturbances together, conversion symptoms involving sexual dysfunction are not coded as Conversion Disorder, but rather as Psychosexual Dysfunction.

Some **psychogenic pain** can be conceptualized as a conversion symptom; but because of the different course and treatment implications, all such cases should be coded as Psychogenic Pain Disorder.

In **Hypochondriasis** typically there are physical symptoms, but there is no actual loss or distortion of bodily function.

In **Factitious Disorder with Physical Symptoms**, the symptoms are, by definition, under voluntary control; and the simulated illness rarely takes the form of neurological symptoms that are likely to be confused with conversion symptoms. However, distinguishing conversion seizures from seizures as a manifestation of Factitious Disorder is often extremely difficult.

In **Malingering** the symptom production is under the individual's voluntary control and is in pursuit of a goal that is obviously recognizable given the individual's environmental circumstance; this goal frequently involves the prospect of material reward or the avoidance of unpleasant work or duty.

Diagnostic criteria for Conversion Disorder

- A. The predominant disturbance is a loss of or alteration in physical functioning suggesting a physical disorder.
- B. Psychological factors are judged to be etiologically involved in the symptom, as evidenced by one of the following:
 - (1) there is a temporal relationship between an environmental stimulus that is apparently related to a psychological conflict or need and the initiation or exacerbation of the symptom
 - (2) the symptom enables the individual to avoid some activity that is noxious to him or her
 - (3) the symptom enables the individual to get support from the environment that otherwise might not be forthcoming
- C. It has been determined that the symptom is *not* under voluntary control.
- D. The symptom cannot, after appropriate investigation, be explained by a known physical disorder or pathophysiological mechanism.
- E. The symptom is not limited to pain or to a disturbance in sexual functioning.
- F. Not due to Somatization Disorder or Schizophrenia.

307.80 Psychogenic Pain Disorder

The essential feature is a clinical picture in which the predominant feature is the complaint of pain, in the absence of adequate physical findings and in association with evidence of the etiological role of psychological factors. The disturbance is not due to any other mental disorder.

The pain symptom either is inconsistent with the anatomic distribution of the nervous system or, if it mimics a known disease entity (as in angina or sciatica), cannot be adequately accounted for by organic pathology, after extensive diagnostic evaluation. Similarly, no pathophysiological mechanism accounts for the pain, as in tension headaches caused by muscle spasm.

That psychological factors are etiologically involved in the pain may be evidenced by a temporal relationship between an environmental stimulus that is apparently related to a psychological conflict or need and the initiation or exacerbation of the pain, or by the pain's permitting the individual to avoid some activity that is noxious to him or her or to get support from the environment that otherwise might not be forthcoming.

Associated features. Psychogenic Pain Disorder may be accompanied by other localized sensory or motor function changes, such as paresthesias and muscle spasm. There often are frequent visits to physicians to obtain relief

despite medical reassurance (doctor-shopping), excessive use of analgesics without relief of the pain, requests for surgery, and the assumption of an invalid role. The individual usually refuses to consider the role of psychological factors in the pain. In some cases the pain has symbolic significance, such as pain mimicking angina in an individual whose father died from heart disease. A past history of conversion symptoms is common. Histrionic personality traits (see p. 313) are seldom present, nor is "la belle indifférence," though concern about the pain symptom is usually less intense than its stated severity. Dysphoric moods are common.

Age at onset. This disorder can occur at any stage of life, from childhood to old age, but it seems to begin most frequently in adolescence or early adulthood.

Course. The pain usually appears suddenly and increases in severity over a few days or weeks. The symptom may subside with appropriate intervention or termination of a precipitating event, or it may persist for months or years if reinforced.

Impairment. This varies with the intensity and duration of the pain and may range from a slight disturbance of social or occupational functioning to total incapacity and need for hospitalization.

Complications. The most serious complications are iatrogenic; they include dependence on minor tranquilizers and narcotic analgesics and repeated, unsuccessful, surgical intervention.

Predisposing factors. Severe psychosocial stress is a predisposing factor.

Prevalence. No information, although the disorder is probably common in general medical practice.

Sex ratio. The disorder is more frequently diagnosed in women.

Familial pattern. Relatives of individuals with this disorder have had more painful injuries and illnesses than occur in the general population.

Differential diagnosis. The dramatic presentation of organic pain, which may seem excessive to an observer because of only slight physical findings, is not sufficient for diagnosing the disorder, and may be only a function of histrionic personality traits or a cultural style of communication.

Individuals with **Somatization Disorder**, **Depressive Disorders**, or **Schizophrenia** may complain of various aches and pains, but the pain rarely dominates the clinical picture, and Psychogenic Pain Disorder should not be diagnosed if the pain is due to any other mental disorder.

In **Malingering**, the symptom production is under the individual's voluntary control, and is in pursuit of a goal that is obviously recognizable given the

individual's environmental circumstances. For example, an individual with Opioid Dependence complains of pain in order to obtain opioids.

The **pain associated with muscle contraction headaches** ("tension headaches") is not to be diagnosed as Psychogenic Pain Disorder because there is a pathophysiological mechanism that accounts for the pain.

Complete disappearance of pain through suggestion, hypnosis, or narcoanalysis suggests Psychogenic Pain Disorder. Temporary improvement due to suggestion has little diagnostic value since it may also occur in true physical illness.

Diagnostic criteria for Psychogenic Pain Disorder

A. Severe and prolonged pain is the predominant disturbance.

B. The pain presented as a symptom is inconsistent with the anatomic distribution of the nervous system; after extensive evaluation, no organic pathology or pathophysiological mechanism can be found to account for the pain; or, when there is some related organic pathology, the complaint of pain is grossly in excess of what would be expected from the physical findings.

C. Psychological factors are judged to be etiologically involved in the pain, as evidenced by at least one of the following:

- (1) a temporal relationship between an environmental stimulus that is apparently related to a psychological conflict or need and the initiation or exacerbation of the pain
- (2) the pain's enabling the individual to avoid some activity that is noxious to him or her
- (3) the pain's enabling the individual to get support from the environment that otherwise might not be forthcoming

D. Not due to another mental disorder.

300.70 Hypochondriasis (or Hypochondriacal Neurosis)

The essential feature is a clinical picture in which the predominant disturbance is an unrealistic interpretation of physical signs or sensations as abnormal, leading to preoccupation with the fear or belief of having a serious disease. A thorough physical evaluation does not support the diagnosis of any physical disorder that can account for the physical signs or sensations or for the individual's unrealistic interpretation of them, although a coexisting physical disorder may be present. The unrealistic fear or belief of having a disease persists despite medical reassurance and causes impairment in social or occupational functioning. The disturbance is not due to any other mental disorder, such as Schizophrenia, Affective Disorder, or Somatization Disorder.

The preoccupation may be with bodily functions, such as heartbeat, sweating, or peristalsis, or with minor physical abnormalities, such as a small sore or an occasional cough. The individual interprets these sensations or signs as evidence of a serious disease. The feared disease or diseases may involve several body systems at different times or simultaneously. Alternatively, there may be preoccupation with a specific organ and a single disease, as in "cardiac neurosis," in which the individual fears or believes that he or she has heart disease.

Associated features. The medical history is often presented in great detail and at length. A history of "doctor shopping" and deterioration in "doctor-patient" relationships, with frustration and anger on both sides, is common. Individuals with this disorder frequently believe that they are not getting proper care. The physical complaints may be used to exert control over relationships with family and friends.

Anxiety and depressed mood and compulsive personality traits are common.

Age at onset. Most commonly the age at onset is in adolescence, although frequently the disorder begins in the 30s for men and the 40s for women.

Course. The course is usually chronic, with waxing and waning of symptoms.

Impairment. By definition there is always some impairment in social or occupational functioning. Social relations are often strained because the individual is preoccupied with disease. There may be no effect on functioning at work if the individual limits the preoccupation with physical complaints to nonwork time. On the other hand, there may be missed work or interference with work efficiency because of the preoccupation. Impairment is severe when the individual adopts an invalid life-style and becomes bedridden.

Complications. Complications are secondary to efforts to obtain medical care. Because of the multiple physical symptoms without organic basis, true organic pathology may be missed. In addition, when the individual goes from doctor to doctor, there is the danger of repeated diagnostic procedures that carry risks of their own, such as exploratory surgery.

Predisposing factors. A past experience with true organic disease in oneself or a family member and psychosocial stressors apparently predispose to the development of this disorder.

Prevalence. This disorder is commonly seen in general medical practice. Because individuals with the disorder are often offended at the suggestion that their fears or beliefs may be unrealistic, they frequently refuse referral for mental health care and are not often seen in mental health facilities.

Sex ratio. The disorder is equally common in men and women.

Familial pattern. No information.

Differential diagnosis. The most important differential diagnostic consideration is **true organic disease**, such as early stages of neurological disorders (e.g., multiple sclerosis), endocrine disorders (e.g., thyroid or parathyroid disease), and illnesses that frequently affect multiple body systems (e.g., systemic lupus erythematosus). However, the presence of true organic disease does not rule out the possibility of coexisting Hypochondriasis.

In some psychotic disorders, such as **Schizophrenia** and **Major Depression with Psychotic Features**, there may be somatic delusions of having a disease. In Hypochondriasis the belief of having a disease generally does not have the fixed quality of a true somatic delusion in that usually the individual with Hypochondriasis can entertain the possibility that the feared disease is not present. The symptoms of hypochondriacal preoccupation may be present in psychotic disorders, in which case the additional diagnosis of Hypochondriasis is not made.

In **Dysthymic Disorder**, **Panic Disorder**, **Generalized Anxiety Disorder**, **Obsessive Compulsive Disorder**, and **Somatization Disorder** the symptom of hypochondriacal preoccupation may appear, but generally it is not the predominant disturbance. In **Somatization Disorder** there tends to be preoccupation with symptoms rather than fear of having a specific disease or diseases. When the criteria for any of these disorders are met and the hypochondriacal preoccupation is due to one of these disorders, the additional diagnosis of Hypochondriasis is not made.

Diagnostic criteria for Hypochondriasis

- A. The predominant disturbance is an unrealistic interpretation of physical signs or sensations as abnormal, leading to preoccupation with the fear or belief of having a serious disease.
- B. Thorough physical evaluation does not support the diagnosis of any physical disorder that can account for the physical signs or sensations or for the individual's unrealistic interpretation of them.
- C. The unrealistic fear or belief of having a disease persists despite medical reassurance and causes impairment in social or occupational functioning.
- D. Not due to any other mental disorder such as Schizophrenia, Affective Disorder, or Somatization Disorder.

300.70 Atypical Somatoform Disorder

This is a residual category to be used when the predominant disturbance is the presentation of physical symptoms or complaints not explainable on the basis of demonstrable organic findings or a known pathophysiological mechanism and apparently linked to psychological factors.

An example of cases that can be classified here include those of individuals who are preoccupied with some imagined defect in physical appearance that is out of proportion to any actual physical abnormality that may exist. This syndrome has sometimes been termed "Dysmorphophobia."